

Crisis And Trauma Research Paper Assignment

Raymond Wagoner

School of Behavioral Sciences, Liberty University

Author Note

Raymond Wagoner

I have no known conflict of interest to disclose.

Correspondence concerning this article should be addressed to Raymond Wagoner. Email:

rdwagoner1@liberty.edu

Abstract

Narrative Therapy is a historically tested and empirically proven theory for treating trauma. The theory has implications beyond trauma, but the focus of this paper is the treatment of trauma. The history of the theory, the history of the school of knowledge, the theory of change, and interventions will all be discussed given the empirically researched efficacy of the theory. The conclusion is that Narrative Therapy and its accompanying interventions are effective for the treatment of trauma.

Keywords: Trauma, Narrative Therapy, Meaning-Making, Narrative Therapy Interventions, Narrative Therapy History

Crisis and Trauma Research Paper Assignment

When selecting a theory of counseling to treat trauma and intervene in a crisis, it is important to be efficient, competent, and trained in the theory. It is also important that the theory is empirically researched for efficacy with the clientele you are working with. When reading about the treatment of trauma in a recent doctoral course on the subject, the author found Narrative Therapy to be both accessible and proven for the treatment of trauma in a variety of settings. What follows is a brief history of Narrative therapy, the creators of the theory, and their aspirations for the theory. There is also a discussion of the current theory, the modern interventions, and the theory of how they create change. A discussion of the evidence of said change will be included as well. As one makes informed decisions about how to treat those affected by trauma, one must address the questions found in this paper.

History

In the 1980's David Epston and Michael White began a search into what causes psychological pathology (Madigan, 2019). The search, according to Madigan, did not begin with a psychological theory in mind, it began with a curiosity by White into a new way to report what he was seeing in clients, a new way of telling their story. As this approach broadened Epston was writing stories to create this new language. The two combined efforts in the late 80s to formally begin work on a theory of psychology that identified the problem a client was carrying with them and ways to reframe the problem in positive terms so the client could effectively wrestle with their pathology through external sources of input (Madigan, 2019). As research and practice progressed, the theory gained a following and became more than a way to tell stories. Eventually, the creators and colleagues turned the theory into a formed methodology of practice we will see below.

Theory of Knowledge

Narrative Therapy (NT) is born of a social construct model (Dallos, 2022; Shakeri, et al., 2020). This means that the authors of narrative theory leaned away from realism and the thought that science held all the concrete answers of the world. Yet, Liebrucks (2001) argued that psychological constructs of a social nature, though they may not be able to use scientific methods to derive knowledge, are not directly incompatible with realism. The idea that we create the world in which we live (Strauss, 2009) contrasts with the traditional theological view of the Judeo-Christian thought that God is the creator and sustainer of the world we live in. However, Kwok (2016) argues effectively that a person's story does not have to be overly individualistic or removed from the greater narrative of the Trinitarian God. Rather, as therapists, we can use a person's narrative to ground them in the story of creation and redemption gifted to us by the Sustainer of the Universe. It is not the narrative that is in opposition to Christian theology, it is the frame of reference given to the client. Thus, NT is compatible with many worldviews and ideologies given the intrinsic nature of one's story. The Judeo-Christian worldview does not have a corner on meta-narratives, nor do the social constructionists have a corner on how another's view of us can change the direction (or plot) of this story.

Theory of Change

Todd May (2005) expresses that our lives are not made up of singular disconnected moments, but our chronology is unfolding from the time we are born. Our stories represent the meaning our lives hold, and the value in the person. But when those stories go unheard or unreported, the meaning remains elusive. Narrative therapy is based on the idea that people make meaning of the world, who they are, and who they are in relation to others through relationships (Madigan, 2019). Using the "landscape of action" (the explicit story) and the "landscape of

consciousness” (the implicit story or meaning) the therapist helps the client re-story their meaning and value (Madigan, 2019). Using questions designed to elicit a new and yet undiscovered version of the story, the therapist helps clients see instances of different emotional responses (i.e., times I didn’t feel left out were...) (Madigan, 2019). Then, mapping the influences in their lives, therapists help clients become authors or co-authors of their own stories (Madigan, 2019). The basis of the theory is that clients have a core problem or destructive behavior that is tangled in their story, and they are unable to make any meaning besides the historical view of themselves and others (PositivePsychology.com, 2023). Using the varied interventions discussed below, a therapist helps untangle the meaning of the story and thereby resolves the problem in a way never conceived before by the client. The theory alone makes sense, but it does not stand on anecdotal evidence alone.

Efficacy

The empirical evidence of efficacy for NT is abundant. There are many studies with diverse populations and symptomology. One study with narrative group therapy proved effective in reducing depression and anxiety in recovering amphetamine addicts (Shakeri, et al., 2020). This interesting study among Iranian inpatients included randomized trials with 26 individuals. The study, divided into groups treated with NT and groups treated with other traditional methods, utilized a two-test measure for efficacy. When the Beck depression inventory-II was administered before and after treatment with the two modes, the group treated with NT showed a statistically significant reduction in depression and anxiety. Although this study was outside the culture in question, the results are significant enough to warrant further research into the effectiveness of NT with addictions and other maladaptive behaviors. Another study conducted by Foroushani & Foruzandeh (2015) showed NT to effectively increase the mental health of

addicts' wives. When the Goldberg General Health Questionnaire was administered to two groups of women (15 in the experimental NT group and 15 in the control group) there was a significant increase in the overall affect of these women. The conclusion is that the problem being the problem is not the problem. The ability to see oneself from an outside editor allows individuals to re-author their narrative based on new information and become a healthier view of themselves. Although Etchison and Kleist (2000) were admittedly working with a scarcity of research on the effectiveness of NT with families, their meta-analysis showed NT to be effective for family therapy issues of many types.

Of the most empirical iterations of NT Skills Training in Affect Interpersonal Regulation (STAIR) narrative therapy provides effective therapy for PTSD and social-related trauma experiences (Cloitre & Schmidt, 2022). I find the interpersonal regulation aspect of this method correlates to and corresponds to Interpersonal Neurobiology. Dan Siegel (2012) reminds us that a traumatized mind is operating outside its window of pain tolerance. Unless a client is helped to soothe this disintegrated mind, they will stay in a state of despair and unable to heal the trauma wounds. Cloitre and Schmidt (2022) agree that emotional regulation must come prior to trauma work and STAIR helps add the skills necessary to do so. STAIR is proven effective in creating identities in clients separate from those entrenched in their past trauma (Erbes et al., 2014). In this study, 14 individuals who met the criteria were entered into a voluntary study for a 12-session treatment program of NT. Several measures including CAPS, BDI-II, and PCL-M were used to evaluate pre-treatment/post-treatment values. The conclusion of Erbes, et al. was that NT is an effective brief therapy for treating PTSD. A quick search demonstrates STAIR has wide accessibility to training and education through government, military, and private organizations.

Interventions

The interventions created by Epston and White and other clinicians along the way are what make NT distinctive from the traditional probing of psychotherapy. Rather than query the client for clues to psychopathology and answers to the why of the behavior or thoughts, NT uses interventions that lead the client to the meaning or the sense-making of their narrative. Most of the questions of NT are designed to scaffold the client into this meaning-making (Madigan, 2019).

Unique Outcome Questions

For example, *unique outcome questions* intervene in the dominant problem in the client's story. As the dominant problem is seen as the reason the client continues in problem behaviors unbeknownst to them, bringing insight into this phenomenon is central to the theory of change. Questions such as "Can you imagine a time in the future when you might defy worry and give yourself a bit of a break?" allow the client to re-story their future without the dominant problem's nonconscious control.

Unique Account Questions

Furthermore, *unique account questions* place the dominant problem in the past and allow for continued co-authoring of the client's narrative and bring the agency back to the client. A therapist might ask "Given everything worry has gotten you into, how do you protest it pushing you around?" to anthropomorphize the problem and show the client that they have more control over how they see themselves and how they behave than they have thought in the past.

Unique Description Questions

Continuing the scaffold, a therapist will then move clients into *unique description questions* building on the above to gain insight into what their story has the potential to develop

into. Introducing a new plot, the therapist asks questions such as “What does this tell you about yourself that you otherwise would not have known?” As meaning-making is the goal of questions, the unique description question allows the client to see how the dominant problem has shaped their thinking over time.

Consulting the Consultant Questions

Finally, a therapist might move a client into a *consulting the consultant question*. Here the therapist is moving the client further into holding agency for themselves and giving them editorial insight into how they can co-author and re-story their narrative. “Given your expertise in the savage ways of anorexia, what have you learned about its practices that you might want to warn others about?” would be an insightful question that leads to meaning-making and ways to have other parts of themselves call the shots other than the dominant problem.

These therapeutic questions are delivered over time and through time as the client starts to recognize the problem is not the problem after all. The client can start to make meaning in interpersonal interactions rather than in isolation of their nonconscious mind. An added benefit of this type of intervention seems to be the regulated way in which the therapist can ask these questions. They are not delivered in a frenzied reaction to maladaptive thoughts or behaviors, rather, they are delivered in a calculated response to the client's narrative. Recognizing that this narrative is built on experience, traumatic events, and clandestine views of oneself, the therapist can gently lead clients into a new plot for themselves.

Letter Writing

One other common intervention with NT is a letter-writing campaign (Madigan, 2019). This intervention is rather lengthy and time-consuming but could prove inspirational to clients struggling with the dominant problem. The first step is to interview the client about people who

might have an alternative view of their story and them as a person. These people would be carefully chosen as those who are influential to the client and would bring either a positive account of the client or provide unseen reasons for the client's dominant problem to exist. For example, an uncle who witnessed early abuse by a father not remembered by the client could offer a concrete reason the client is fearful of deep male relationships. Once the participants are chosen, the therapist will send out invitations to answer specific questions about the client. Again, questions designed to add new insight or a positive reflection of the client are chosen. No one is asked for an extemporaneous account of their own making. The therapist is acting as an editor, carefully choosing what content adds to insight and meaning-making for the client. After all the letters are received back from the participants, respondents' letters are then read and witnessed in the presence of the client. Each respondent being present to read their own letter is a complication of this intervention. I believe that the use of virtual meetings could streamline this process and bring respondents who would have been disqualified in the past. Of course, the therapist would read these letters first to ensure they are helpful to the process and nothing re-traumatizing or surprising to the process would be read without prior knowledge of how to debrief. Debriefing is the next step in the intervention. Allowing clients to respond to what they have heard and add to or edit their narrative is important to the process. If a client is not allowed to process interpersonally what they have witnessed, they will simply revert to the nonconscious problem of old. The theory behind this intervention is that clients hear a different narrative and can co-author a changed view of themselves called re-membering. The new information makes new meaning and makes sense of why they have been thinking, feeling, and behaving one way for a lifetime.

Re-story

Other interventions common to NT include a *Re-story*. This intervention allows the client to find new ways to make alterations to the meaning of their experiences (PositivePsychology.com, 2023). Many times client's narrative is built from the outside. It is built from what people have told them, implicit memories that include details they don't understand, or traumatic events that have them frozen in fear. Re-storying allows the client to find their voice, and to tell their story from their perspective without judgement or criticism. This intervention can and should be used regardless of one's theoretical approach. The ability of a person to speak and be listened to and acknowledged cannot be underestimated.

Externalization

Similar to Re-storying is *Externalizing*. Externalizing is simply separating oneself from one's problems. This intervention key to and created by NT is important in changing one's view of oneself from being completely self-created to being a part of society or humanity. A key concept of this intervention is calling the client to change behavior rather than changing personality (PositivePsychology.com, 2023). It is a first-order change in personality divergence, but behavior can be monitored and managed much more routinely. Helping clients connect behavior to the dominant problem and viewing both as outside their core desires seems more like Cognitive Behavioral Therapy. However, in connection to the story aspect of NT one can see that it is an empowering idea for connecting maladaptation to health.

Deconstructing the Problem

Deconstructing the problem or listening for meaning and searching for alternative meanings to the story is a job for the therapist (Jagatdeb et al., 2024; PositivePsychology.com, 2023). As a therapist listens to a client's narrative and curiously seeks clarification of emotions and thoughts, the therapist then offers insights the client has missed or dismissed as unimportant.

By offering alternative meanings to the narrative (similar to the letter-writing champagne) the therapist can help the client reimagine the problem as something that can be avoided or accepted differently. This intervention could turn into a directed model if not careful. Directed models are not intrinsically bad, but they are not in keeping with the theory of NT. Giving the client rule and authority over their own story is woven into the fabric of NT. An intervention in NT must be client-centered and non-directive. Offering insight in the form of probing questions or “what-ifs” would be the best way to offer this intervention.

Mapping

Lastly, and similar to deconstructing the problem is the intervention of *Mapping*. This again is the role of the therapist as the client shares their story in more and more detail and depth. A client who is progressively adding implicit memories to their explicit memories will aid the therapist in mapping their journey. In mapping, the therapist is listening for where the problem intersects multiple aspects of life and searching for both new behaviors and new meanings of the problem (PositivePsychology.com, 2023). Adding chapters and books to the language of mapping can be helpful. If a client can see their behavior in one season as just a chapter in their story, shame is often diminished. Adding books to the story allows the client to have a sense of new meaning and new behaviors as they accept the past and look for who they are through a different lens (their own or God’s rather than everyone else’s). Mapping helps connect the implicit to the explicit as well. Many of us hold thoughts, and beliefs, and continue behaviors based on non-factual information. Our systems are asking us to respond to a yet unresolved threat and we become stuck in the same pattern of response. If a therapist can non-directivity shape the insight of how a client’s experiences have them trapped, there is potential for the client to regulate that threat in a new way.

These and other interventions are proven ways to help clients recall and re-story and re-memory their narratives. Through clever articulation of the client's view of themselves, God, and the world, a therapist can help them become the co-author of their story. Each intervention mentioned is carefully tailored to give the client agency and clarity in dealing with how the past intersects with the present. I typically prefer externalizing because it is a way to help clients identify with their true selves.

Conclusion

As the past intersects with the present, trauma work is done. It is no surprise that NT is effective in creating change in clients affected by trauma. As shown, whether a client's trauma is self-inflicted due to addictions or inflicted by others, one goal of NT is to reduce the feelings of being a victim and increase the feelings of autonomy and one's ability to speak for oneself (PositivePsychology.com, 2023). It is clear that NT is an empirically proven, effective treatment for trauma when the interventions of the theory are carried out by qualified clinicians. Training, education, and mentoring for NT practice are readily accessible. The history, methodology, and practice of NT are evidence in themselves that many with trauma-based maladaptive behaviors and thoughts are healthier today because of the theory. The implications for clinical work are far-reaching and even the interventions separate from the theory are effective for creating change in depression, anxiety, and maladaptive behaviors.

References

- Cloitre, M. & Schmidt, J. A. (2022). STAIR Narrative Therapy. In *Evidence based treatments for trauma-related psychological disorders: a practical guide for clinicians* (pp. 307–328). Springer. https://doi.org/10.1007/978-3-030-97802-0_14
- Dallos. (2022). Attachment Narrative Therapy. In *Attachment narrative therapy: applications and developments* (pp. 1–31). Palgrave Macmillan. https://doi.org/10.1007/978-3-031-12745-8_1
- Erbes, C. R., Stillman, J. R., Wieling, E., Bera, W., & Leskela, J. (2014). A Pilot Examination of the Use of Narrative Therapy with Individuals Diagnosed with PTSD. *Journal of Traumatic Stress*, 27(6), 730–733. <https://doi.org/10.1002/jts.21966>
- Etchison, & Kleist, D. M. (2000). Review of Narrative Therapy: Research and Utility. *The Family Journal*, 8(1), 61–66. <https://doi.org/10.1177/1066480700081009>
- Foroushani, E. Z., & Foruzandeh, E. (2015). Effectiveness of narrative therapy on mental health of addicts' wives treated in the addiction treatment centers in Khomeini Shahr. *International Journal of Educational and Psychological Researches*, 1(4), 266.
- Jagatdeb, S., Mishra, S., Bajpai, T., & Sen, S. (2024). “Externalizing the Internalized:” Exploring Externalizing Conversations in Narrative Therapy with Adolescents and Young Adults in the Indian Context. *Journal of Indian Association for Child and Adolescent Mental Health.*, 20(2), 153–160. <https://doi.org/10.1177/09731342241238096>
- Kwok, W. (2016). Narrative Therapy, Theology, and Relational Openness: Reconstructing the Connection Between Postmodern Therapy and Traditional Theology. *Journal of Psychology & Theology*, 44(3), 201–212.
- Liebrucks. (2001). The Concept of Social Construction. *Theory & Psychology*, 11(3), 363–391.

<https://doi.org/10.1177/0959354301113005>

Madigan, S. (2019). The therapy process. In *Narrative therapy* (2nd ed., pp. 63–108). American Psychological Association. <https://doi.org/10.1037/0000131-004>

May, T. (2005). *Gilles Deleuze: an introduction*. Cambridge University Press.

<https://doi.org/10.1017/CBO9781139165419>

PositivePsychology.com. (2023). *19 Best Narrative Therapy Techniques & Worksheets*.

<https://positivepsychology.com/narrative-therapy/>

Siegel, D. (2012). *Pocket Guide to Interpersonal Neurobiology: An Integrative Handbook of the Mind*. W. W. Norton.

Shakeri, J., Ahmadi, S. M., Maleki, F., Hesami, M. R., Moghadam, A. P., Ahmadzade, A., Shirzadi, M., & Elahi, A. (2020). Effectiveness of Group Narrative Therapy on Depression, Quality of Life, and Anxiety in People with Amphetamine Addiction: A Randomized Clinical Trial. *Iranian Journal of Medical Sciences*, 45(2), 91–99. <https://doi.org/10.30476/IJMS.2019.45829>

Strauss, D. F. M. (2009). The (Social) Construction of the World – at the Crossroads of Christianity and Humanism. *South African Journal of Philosophy*, 28(2), 222–233.